

Imported external MR for MDT meeting IEP**Clinical information:**

Primary diagnosis of a moderately differentiated adenocarcinoma of the rectum. External MRI report is not available

Findings:

The primary tumour is demonstrated as an annular mass with a infiltrating border.
The distal edge of the luminal tumour arises at a height of 40mm from anal verge.
The distal edge of the tumour lies 0mm above the top of the puborectalis sling.
The tumour extends craniocaudally over a distance of 64mm.
The tumour has a maximum thickness of 26mm
The proximal edge of tumour lies at a vertical distance of 17mm below the peritoneal reflection
The invading edge of tumour extends from 9 to 3 o'clock.
Tumour is extends through the muscularis propria with prostate invasion anteriorly
Extramural spread is 16mm.

MR T stage: T4 visceral .

Tumour is present at the distal levator level.

Tumour extends into adjacent prostate : exenterative procedure will be required.

Lymph nodes assessment:

Vascular deposits : N1c

Extramural venous invasion:

Clear and definite irregular expansion of vein. Large vein invasion is present, affecting the inferior and middle rectal veins

CRM:

Closest circumferential resection margin is at 12 o'clock.

Closest CRM is from direct spread of tumour .

Minimum tumour distance to mesorectal fascia:0mm CRM involved.

Peritoneal deposits: No evidence .

Pelvic side wall lymph nodes: Malignant Mixed signal/irregular border

Location: a solitary R internal Iliac vascular deposit

Opinion:

MRI Overall stage: T4 visceral N1c Mx CRM involved EMVI positive PSW positive

Eligible for the TRIGGER trial , SERENADE and beyondTME

Examination: :Imported external MR for MDT meeting IEP

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